

Headache

Source: National Institute of Neurological Disorders and Stroke (NINDS), National Institutes of Health. Last reviewed March 13, 2026. U.S. Government work, public domain.

Headache is a condition that's one of the most common causes of pain and a major reason people miss work or school — and visit the doctor. Talk with a doctor if headaches affect your daily life.

Headaches vary in how often they happen and how much pain they cause. Some headaches may happen repeatedly (episodic) or last for days or weeks at a time (chronic). Pain can range from mild to severe and may come with symptoms such as nausea or increased sensitivity to noise or light.

There are three groups of headache. Primary headache happens on its own, while secondary headache happens because of some other medical condition. A third group includes headache from nerve damage or facial pain that doesn't fit under primary or secondary headache.

Tension-Type Headache

Tension-type headache is the most common type of headache condition. The “tension” in the name points to how emotional stress is an important trigger for this kind of headache — stress can lead muscles in the neck, face, scalp, and jaw to contract. Other common triggers include mental strain, not enough or poor-quality sleep, physical postures that strain the head or neck (such as reading, working on a computer, or looking at a phone screen), alcohol use, dehydration, and sunlight exposure.

The pain is usually mild to moderate and feels like constant pressure on the face, head, or neck, often on both sides of the head — it may feel like a belt is being tightened around the head. Unlike migraine, there's no pre-headache aura, nausea, or vomiting, and it doesn't get worse with everyday physical activity like walking or climbing stairs. Tension-type headaches can last from 30 minutes to 7 days; the chronic form happens more often and can last from hours to days, or be constant.

Migraine

Migraine is a health condition where a person has repeated attacks of moderate to severe throbbing and pulsating pain, usually on one side of the head. It happens in both children and adults, and affects women more often than men.

Cluster Headache and Related Conditions

Trigeminal autonomic cephalalgias (TACs) are rare conditions where pain happens on one side of the head, along with symptoms like eyelid swelling, nose congestion, and sweating. The most common and often most severe type is cluster headache: sudden, extremely painful attacks usually at the same time of day or night for several weeks at a time (a “cluster period”), with one to several attacks a day. Pain is usually on one side of the head, often behind or around one eye, peaking 5 to 10 minutes after starting and continuing at that intensity for up to three hours. The nose and eye on the affected side may get red, swollen, and teary. Cluster headache generally begins between ages 20 and 40, is more common in men than women, and is more common in smokers — alcohol (especially red wine) and smoking can trigger an attack.

Secondary Headache

Secondary headache is a symptom of another health condition that presses on, pulls, or pushes pain-sensitive nerve endings. Causes include:

- Blood vessel disorders in the brain, including stroke
- Brain tumor or brain injury
- A change in routine, like air travel or fasting
- High blood pressure
- Seizures
- Substance misuse or withdrawal
- Infection

Who Gets Headaches, and What Triggers Them

Anyone can experience a headache, and certain headache conditions run in families. Foods and drinks that may trigger a headache attack, especially migraine, in some people include aged cheeses, aspartame, caffeine (or caffeine withdrawal), chocolate, cured or processed meats, MSG, salted nuts, and wine or other alcohol. Other triggers include emotional stress, weather changes, changes in eating and sleep patterns, and dehydration.

When to Call or See a Doctor Right Away

- A sudden, severe headache, possibly with a stiff neck
- A severe headache with fever, nausea, or vomiting not related to another illness
- A “first” or “worst” headache that also comes with confusion, weakness, double vision, or loss of consciousness
- A headache that gets worse over days or weeks, or changes in pattern
- A headache after a brain injury
- A headache with loss of sensation or weakness in any part of the body — this could be a sign of a stroke
- A headache with convulsions or trouble breathing
- Two or more headaches a week
- A constant headache in someone who hasn't had headaches before, especially if they're over age 50
- New headaches in someone with a history of cancer or HIV/AIDS

Diagnosis

A doctor will review a person's medical and headache history and do physical and neurological exams, and may order tests — blood, urine, or fluid tests, a CT scan, an MRI, or an EEG — to look for a specific cause. Keeping a headache journal (noting time of day, intensity and duration, sensitivity to light or

sound, activity beforehand, medicines taken, sleep the night before, stress, weather, and food and drink in the prior 24 hours) can help a doctor identify patterns and reach a diagnosis.

Treatment

Headache treatment is a partnership between the person having headaches and their doctor, and it may take time to find what works. The first step is identifying and treating any underlying health condition that could be causing the headaches. Over-the-counter medicines like aspirin, ibuprofen, or acetaminophen can help with pain; a doctor may also recommend prescription options depending on the type of headache — for example, triptans for migraine, or verapamil to help prevent cluster periods. Regularly overusing headache medication can actually worsen headaches or cause new headache symptoms, so a doctor can help build a plan for safe use.

Biofeedback, relaxation training, meditation, and cognitive-behavioral therapy can help people manage stress from headache. Getting enough sleep (too little or too much can both worsen headaches), regular exercise, and a healthy diet can also help prevent them.

For More Information

- American Headache Society (AHS) — 856-423-0043
- American Chronic Pain Association (ACPA) — 913-991-4740
- National Headache Foundation — 312-274-2650 — www.headaches.org

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